



Occupational Medicine Cases

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Fall 2025

Case # 1- Deputy Eastwood

- Anywhere County Human Resource Department sent you a request to evaluate Deputy Sheriff Eastwood for a Fitness For Duty exam. The County is concerned of Deputy Eastwood's abilities to continue as a detention sheriff given recent events. He has been relieved by the County of his peace officer powers and the County has retained his service weapon. A week ago, the County was notified by the local City Police Department that Deputy Eastwood attempted suicide by wanting to jump off a ledge. A 911 call was recorded, and Deputy Eastwood could be heard yelling to his wife that it was "his time to go". Fortunately, his wife was able to convince him to come off the balcony. The City Police Department was called, and the Deputy was taken to the County Hospital for a 72-hour hold. After being released from the County Hospital, Deputy Eastwood told his commanding officer that he was placed on the 72-hour hold because they wanted to observe him since his blood alcohol level was 0.3% and they wanted to make sure he did not go into withdrawal. He stated that the doctors did not find any mental health problems. Deputy Eastwood said he followed up with personal doctor who gave him a note that stated, "Mr. Eastwood denies any history of mental health problems currently, and he is fit to return to work."



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No

No

Questions:

psych eval, mandatory substance abuse training, rehab, counseling

sud/aud, access to firearm, spousal abuse

yes

Is Deputy Eastwood safe to return to his usual and customary job?

Do you accept the doctor's note of clearance?

How would you assess Deputy Eastwood's fitness for duty?

What are other potential issues with Deputy Eastwood?

Are there legal risks to the public and to the County?

Case #2 – Mr. Law

- ♦ Mr. Law is a 22-year-old cis-gender male who is applying to attend the police academy with the intention that he will be hired on to the City Police force after graduating. Mr. Law is sent to your office for a pre-employment medical evaluation. Mr. Law has a history of type 1 diabetes and he manages the diabetes with insulin. He does a great job of monitoring his blood sugars daily and he presents a log showing the ranges from 70-200 mg/dl. His last visit to the endocrinologist was a month ago and his Hb A1C shows 7.3% and normal kidney and liver functions.



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Questions

- ♦ In addition to the above lab findings, what other diagnostic studies (if any) would you want prior to clearing him for the police academy? fasting glucose, cgm, pump?
- ♦ What restrictions (if any) would you place on the patient before clearing him for the police academy (based on his type 1 DM and the rigorous expectations)? requiring A1C regular checks, mandatory meal times, cgm, pump
- ♦ How would the presumptions associated with police work and long-term diseases be affected by his type 1 diabetes at the time of hire?

whether he keeps it under control otherwise will lead to other systemic complications

Case #3 – Ms. Mailperson



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- ♦ Ms. Mailperson is a healthy 56-year-old cis-gender female postal carrier for the past 20 years. She has no significant past medical history, but started to develop shortness of breath, dizziness, wheezy cough, and fatigue over the past 10 months. She noticed her symptoms began when she changed her postal route about the same time. Her new route included delivery to the rural parts of town including several chicken farms. Days when she did not work, she felt better. Ms. Mailperson had a complete work up by her primary care physician and was diagnosed with an upper respiratory infection based on the radiologist report of “slight diffuse greyness to the lungs.”



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Questions:

- ♦ Is this work-related? [yes](#)
- ♦ What other questions would you ask to determine if this is work related? [when did sx start?](#)
- ♦ What recommendations would you make for work restrictions if any? [change routes, wear a respirator](#)

Case #4: Ms. Tremors

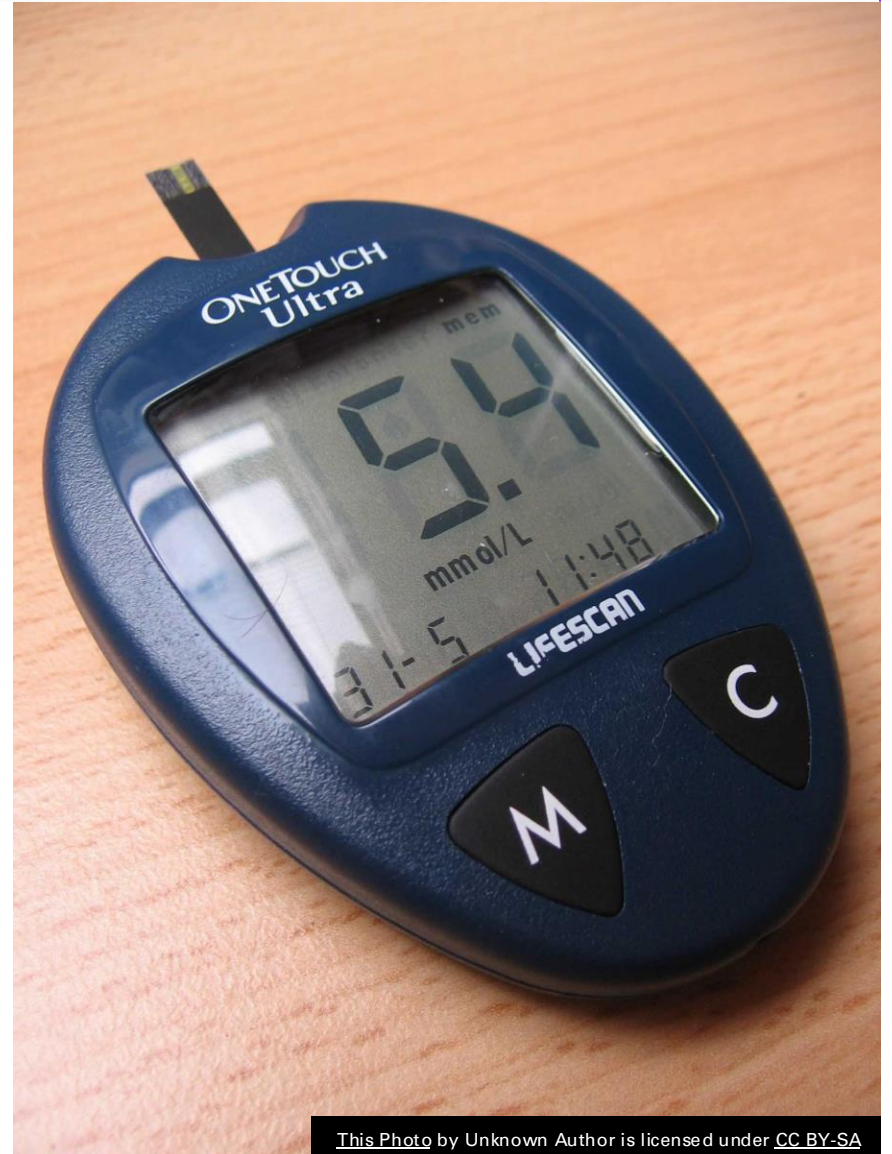
- ♦ Ms. Tremors is a 33-year-old cis-gender female who suffered a grand mal seizure at work. She works as an office worker and was sitting at her desk when she had the seizure. She hit her head on the desk and fell off the chair and fell to the floor, which caused a contusion of her forehead and a thoracic strain. She was taken to the ER after the incident and was determined to have the seizures because of low blood sugars. Ms. Tremors was sent to you to evaluate for a work-related injury and treatment of her injuries.



Questions:

- ♦ Is this work related? [yes](#)
- ♦ What recommendations would you make for work restrictions if any? [mandatory meals, sugar check](#)
- ♦ Is the employer responsible for having the patient follow up with hypoglycemia?

[yes](#)



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